

THE LIVED PREGNANCY EXPERIENCE OF WOMEN IN PRISON

Judith Merenda Wismont, RN, PhD

ABSTRACT

The professional literature reflects limited awareness of and/or concern for the effects of incarceration on the physical and psychological dimensions of childbearing. The purpose of this study was to describe the childbearing experience as reported by pregnant incarcerated women. Giorgi's phenomenological method was used. Apprehension, grief, subjugation, and relatedness were the essential themes related to the experience of childbearing in prison which emerged from the analysis of the transcripts of the journal entries and interview data. Incarceration was the fulcrum on which the women's pregnancy experiences balanced.

Rapidly rising numbers of women imprisoned in the United States underscore the urgency to better understand their experiences. Health care providers must find ways to help female inmates cope more effectively with the role diffusion/confusion they experience as they live two diametrically opposing roles—those of inmate and mother. Correctional facilities must also be assisted in establishing policies that recognize the unique needs of pregnant incarcerated women. *J Midwifery Womens Health* 2000;45:290–8 © 2000 by the American College of Nurse-Midwives.

Pregnancy is generally a period of personal fulfillment and eager anticipation of the birth of a new child. Pregnant incarcerated women, however, are subjected to a variety of policies that negatively affect their pregnancy experiences. Most significantly, almost without exception, these women are separated from their newborns at the end of the routine obstetric hospitalization.

Generally, women who give birth while in prison are pregnant at the time of their incarceration. Since the median prison time served for women is 12 months (1), most pregnant prisoners will spend between 6 months to a year in prison after the birth of their children.

The U.S. Department of Justice reported that in 1999, 5% of women remanded to state prisons were pregnant. Extrapolating from the data available on women prisoners held in state and federal prisons in 1997 and 1998 (2,3), 4,803 women were admitted to prison in 1998. If 5% of these women were pregnant when beginning their sentences, then approximately 240 women entered prison pregnant.

Pregnancy and early postnatal experiences are believed to be of critical importance for the development of

maternal identity (4) as well as the enhancement of the emotional well being of the maternal/child dyad (5). Pregnant incarcerated women know that unless paroled, they will return to prison without their newborns within (at most) several days of birth. How does the knowledge of this impending separation affect these mothers' pregnancy experiences and, subsequently, the attachments they form with their fetus/newborn?

The purpose of this study was to describe the lived experience of maternal-fetal attachment as told by 12 pregnant incarcerated women.

LITERATURE REVIEW

Maternal-Fetal Attachment

The earliest descriptions of a woman's relationship with her unborn child had their origins in psychoanalytic theory and were primarily the result of observations of pregnant women in therapy. Even so, early theorists (6,7,8) described pregnancy as a developmental stage during which a mother progressively invests in her fetus. In 1984, building on the proposition that pregnancy is a distinct developmental stage, Rubin (9) identified four tasks necessary for its successful completion; these included seeking safe passage, binding-in to the child, giving of one's self, and seeking acceptance for the child. Rubin further suggested that prenatal binding-in subsequently formed the basis for the mother's relationship with her child after birth. Binding-in or attachment was described as an acquaintance process through which the woman comes to know, have affection for, and interact with the fetus as a separate individual during pregnancy. More recently, researchers have found that within the context of uncertainty, pregnant women in high-risk situations also strive to accomplish the developmental tasks of pregnancy identified by Rubin (10).

Maternal-fetal attachment has been conceptualized as engagement in behaviors and/or affiliation with an unborn child (11,12). Whether operationalized by feelings or behaviors, attachment is believed to be a process that begins during pregnancy (9,13,14).

Childbearing in Prison

The literature reflects limited awareness of and/or concern for the effects of incarceration on the physical and psychological dimensions of childbearing (15). Concern

Address correspondence to Judith Merenda Wismont, RN, PhD, University of Michigan, Room 3183 SNB, 400 N. Ingalls, Ann Arbor, MI 48109-0482.

for the quality of general health care for incarcerated women, however, is a common thread in the work that has been done (16–21). Not surprisingly, the literature suggests that prisons have been negligent in meeting the health care needs of pregnant inmates; this in turn has resulted in poorer birth outcomes and more complications for pregnant incarcerated women as compared with those of women who are not in prison. Deficiencies were identified in the areas of routine health and obstetric care, as well as emergency care (16–19,22). Problems identified include off-site prenatal care, inadequate prenatal examinations, lack of childbirth preparation classes, lack of emotional support during pregnancy, and an increased rate of cesarean delivery.

It should be underscored that the significance of pregnancy-related complications and health outcomes for pregnant inmates and their newborns has been debated. Whereas many authors report increased negative health outcomes for pregnant inmates, others find no greater complication rate when incarcerated women are matched with controls on age, parity, race, and entry into prenatal care (23,24).

Psychological dimensions of childbearing in prison are influenced by the prison environment and the distinct psychosocial characteristics of the prison population. Hufft (25) identified increased general stress, environmental restrictiveness, alterations in social support systems, and maternal role displacement as four categories of psychosocial needs particular to pregnant incarcerated women. Similarly, perceptions of inadequate medical care, isolation, stress, fear, maternal role transition, and parenting concerns were commonly repeating themes identified by both inmates and by professional and lay writers (15,19,22,25–27). Anger, regret, and depression regarding their separation from and their inability to care for their infants have been reported (22). Shelton and Gill (22) also found that the women distanced themselves psychologically from their fetuses.

METHOD

Believing that maternal-fetal attachment might be affected by incarceration, the following lines of inquiry were pursued.

1. What is it like for you now to be pregnant and in prison?
2. Some women think about their unborn babies often.

Judith Merenda Wismont received her PhD in 1996 from Wayne State University. She taught prenatal education classes for several years at the regional women's prison on a volunteer basis. Currently, she teaches in the undergraduate maternity course in the School of Nursing at the University of Michigan.

At this time, how do you feel about (relate to) your unborn baby?

3. What is it like for you now knowing that your baby may be away from you after birth?

The first question elicited the contextual aspects of the experience, allowing the participant the opportunity to identify and describe issues related to her pregnancy that she believed to be most significant. The second question specifically focused on her experiences of maternal-fetal attachment. The third question explored the mother's feelings about the impending separation from her newborn.

Design

Because the purpose of this study was to explore the lived experiences of childbearing women in prison, a phenomenological method was used. "The phenomenological method seeks to uncover the meaning of human experienced phenomena through the analysis of subjects' descriptions" (28). Through structured reflection (phenomenological reduction), the essence of human experiences is described with scientific exactness. Through this process, one is able to achieve an awareness of the world with all subjective influences removed. To achieve this exactness, the researcher must first "bracket" or suspend commonly held beliefs. With this achieved, the universal essences of the phenomena being explored emerge from the participants' descriptions. Use of the phenomenological method resulted in rich descriptions of the pregnancy experiences of incarcerated women, which were then analyzed for relevant themes.

Participants

Twelve participants ranging in age from 22 to 36 years were recruited. One woman was pregnant for the first time. Two women were pregnant for the second time; however, their first pregnancies had not been carried to the age of viability for the fetus. The other nine women had each been pregnant between two and nine times. Of these pregnancies, individual participants had birthed between one and three living children (although not all were currently living). Eleven participants intended to raise this child upon their release from prison and one planned to place her newborn for adoption. Seven of the study participants reported their racial/ethnic heritage as black or African American, three were white, one was black/Indian, and another was white/American Indian.

Participants were asked how long they expected to be in prison and what their earliest and latest potential release dates were. Responses about the anticipated length of incarceration ranged from "not long" to the year 2014. Their earliest possible release date often corresponded with their anticipated length of stay. Their

longest release dates, however, indicated that they could serve many more years in prison if denied parole.

Setting

The study was conducted at a midwestern state prison. This maximum security prison currently houses approximately 850 women and is the regional center in the state for pregnant inmates. Fifteen to 20 inmates are pregnant at any given time. The prison is comprised of six resident buildings with security levels ranging from low to maximum. Each building houses approximately 180 women. Pregnant inmates are usually clustered in one moderate security building. The building is divided into two halves, with each half functioning as an independent unit. Each half is further divided into upper and lower levels. Women are restricted in their movements to the level on which they are roomed except when entering or leaving the building. Meals, health services, education, and visits with approved family and friends take place in buildings other than the resident unit. Prisoners are double bunked in rooms designed for one.

Data Collection

Permission for the researcher to tape the interviews with the participants was obtained from the appropriate institutional review boards. When the study began, however, the prison warden told the researcher that rules prohibited the removal of taped interviews from the prison, so an alternative method of recording the data was agreed upon. A hard-bound notebook was given to each participant to serve as a journal in which to record her thoughts and feelings. The lines of inquiry intended to guide the participants' reflections were written inside the journal's front cover. When the journal was given to the participant, the questions were reviewed to clarify any ambiguity. The woman was asked to address each question and to write as little, or as much, as she wished. Informed consent was obtained when the woman agreed to participate in the study.

Each participant was to write in her journal during each remaining trimester of her pregnancy. Because their gestational ages at recruitment varied, seven women made one, one woman made two, one woman made three, and three women made ongoing journal entries (much like writing in a diary). Although these last three women did not directly address the individual questions, their thoughts and feelings relative to the lines of inquiry were clearly evident in their writing.

The researcher transcribed the journal entries and the typed text was reviewed with each participant at individually scheduled interviews. During the interviews, the researcher asked the participants to clarify passages that were not clear and to expand on their entries as they

wished. Saturation was reflected when after the flow of the interview stopped, the women answered "no" to the question, "Is there anything else you would like me to know about how you feel about your baby or about your pregnancy experience in general?" The journal entries combined with the interview information comprised the data that were subsequently analyzed.

Data Analysis

Giorgi's protocol (29,30), which focuses on context related factors, was used for the analysis. Although Giorgi developed his protocol from within the discipline of psychology, his focus on contextually related factors is philosophically congruent with nursing and midwifery. Giorgi's protocol directs the researcher to:

1. Read completely through the narrative to get a sense of the whole.
2. Read through the narrative again and divide the transcript into units (blocks) that seem to express a self-contained meaning from a psychological perspective.
3. State, as simply as possible in one's own words, the meaning that dominates the natural unit. This is the first transformation.
4. Interrogate each meaning unit and its theme in terms of the specific topic of the study. The researcher draws out from each unit of the narrative those aspects that are pertinent and redescribes these aspects in the language from the perspective of psychology. This is the second transformation.
5. Synthesize and tie the themes together into a descriptive statement of essential, non-redundant psychological meanings. The transformed units are now related to each other and to the sense of the whole narrative.
6. Develop a description at the general level from the narrative, leaving out the particulars of the specific situation and centering on aspects that are transsituational or descriptive of the "experience topic" in general. This step may be optional if multiple respondents are involved.
7. If possible, synthesize the transformed meaning units from the various respondents into a final general description.

Trustworthiness of Findings

Reliability of measurement tools and validity of concepts and findings are essential considerations when evaluating quantitative studies. Due to conceptual and methodologic differences, however, these "quality control" measures are not directly applicable to qualitative methodologies. Leininger (31) identified six criteria by which the trustworthiness or scientific rigor of qualitative studies could be determined. These included credibility, confirmabil-

ity, meaning in context, recurrent patterning (consistency), saturation, and transferability.

Credibility refers to the “truth value or to the believability of the findings” (32). Credibility was achieved by accurately describing the experiences expressed by the participants. The actual words of the participants were incorporated into the findings to give credence to the analysis presented.

Confirmability refers to “establishing verifiable direct evidence with the people or from primary documents or experiences” (32). The model summarizing the themes that emerged from the data was shared with four participants still at the prison at the completion of the study. All agreed that both the themes and the model accurately represented their experience. One woman stated: “This exactly describes my life since I’ve been in here.”

Meaning-in-context refers to “data that are understandable and relevant within certain situations, settings, and life experiences” (32). The in-depth exploration provided a rich description of the childbearing experiences of women in prison, enriching the body of existing work in this area.

Recurrent patterning refers to human events, experiences, or lifeways that show identifiable patterns over time (31). The universality of essential themes among 12 participants demonstrated this criterion.

Saturation is having obtained all that can be understood about the phenomenon from the informants (31). It is achieved through exhaustive exploration of the phenomenon. Saturation of data was evidenced by the recurrence of essential themes as well as participants’ verbal affirmation of closure on the topics.

Transferability refers to “whenever the particular findings from a qualitative study can be transferred to another similar context or situation and still preserve the particularized meanings, interpretations, and inferences of the completed study” (33). The findings of this study reaffirm the findings of other research studies which focus on the childbearing experience of women in prison.

RESULTS

Four essential themes related to the experience of childbearing in prison emerged from the analysis of the transcripts of the journal entries and interview data. These themes were apprehension, grief, subjugation, and relatedness. Incarceration was the fulcrum on which the women’s pregnancy experiences were balanced. The model reflecting the relationships among the emergent themes is presented in Figure 1.

Theme 1. Apprehension

The women in this study expressed pervasive feelings of foreboding regarding both fetal well being and estrange-

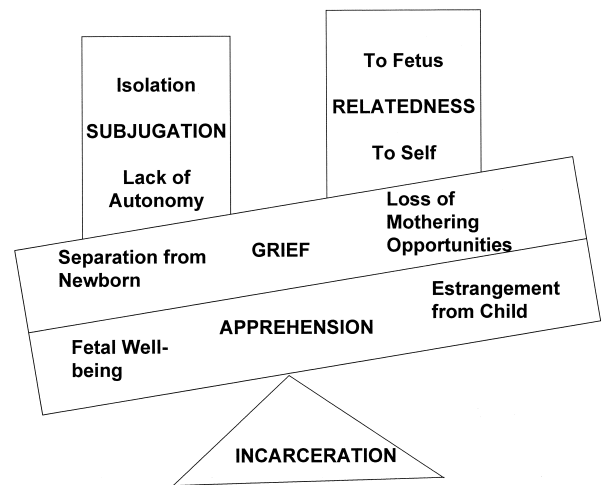


FIGURE 1.

The four themes and related elements that describe the experience of childbearing in prison.

ment from the child. Specific factors they thought exerted a negative influence on fetal well being included inadequate prenatal care, the effects of their use of drugs during early pregnancy, the presence of sexually transmitted diseases, and the effects of maternal stress. Post-birth issues included finding a caregiver for the child and concern that the child’s attachment to the mother might be weakened as a result of the separation.

Lack of choice about their health care provider (HCP), as well as the difficulties and/or delay in being seen for health care (within the prison) when concerns arose were cited by the women as threats to fetal well-being. Another significant threat was the delay by prison authorities in arranging transport to the hospital for the laboring inmates. One mother stated:

If you are having trouble with your pregnancy, they act like they don’t care or it’s not a big deal. I had an incident of vaginal bleeding 2 weeks ago (at 29 weeks gestation). Health Care told me to go back and lay down. When they finally took me to the hospital, I was having contractions.

Another wrote: “I not only think about my unborn child I worry about how it will make it into this world. Will we make it to the hospital on time.” Another said: “They make it real evident they don’t care for me or my baby.”

Concerns about potential dangers to their fetuses associated with risky behaviors during early pregnancy can be heard in the following excerpts: “I was on drugs in the beginning of my pregnancy so I always pray that my baby will come out healthy” and “Well, today after I spoke to you (about syphilis), I felt better. As I said, being a girl who works the streets, you never know what to expect. I just pray that my baby is born happy, most of all healthy.”

Apprehension related to the baby extended beyond birth. The foremost worry of the women was finding a caregiver for their child. They also worried about whether the baby would be harmed by the stress the mother experienced as a result of her incarceration. How this stress might harm the baby was often not clearly expressed, yet this general concern was frequently raised, as demonstrated in the following excerpts: "I know I caused my baby to feel my stress with me crying. I was upset and my baby knotted up" and "I know that it's hard on me so I can't start to imagine what it is like for him."

Participants identified specific stressors in their everyday lives. These included finding a caregiver, dealing with prison officers and rules, and interacting with other inmates. The following passages attest to the stress that one mother experienced as she sought a caregiver. "Also, another fear that I have is there may not be a family member to care for my baby. Therefore, it might be placed in foster care. My sister has told a friend that she will care for this baby. I've been unable to reach my sister" (23 weeks gestation). At 31 weeks gestation, this same woman wrote: "I still haven't heard from my sister. Mom said my sister would care for my baby. I figure she should know because my sister is living with my mom. . . . I don't have my mom's phone number. She refuses to give it to me."

Once a caregiver was found, there was considerable worry that as a result of the separation the child would either not know the mother upon her release, or that during the time of separation the child would form an attachment to someone else. One mother's passage summarized an apprehension that all but the mother planning to place her baby for adoption voiced in one way or another. She wrote: "I also think about will she know me when I get out by it being six months old by my parole date." Finally, mothers expressed concern that the child's perception of the mother would be lessened and that his/her subsequent relationship with her would be detrimentally affected by the knowledge of the mother's incarceration. One mom wrote: "I really feel sorry for my unborn child. Wondering how he will feel after the separation. Will this child forgive me? It's a hurting feeling."

Theme 2. Grief

With the exception of the woman planning to place her child for adoption, all of the participants, regardless of weeks of gestation when writing, expressed profound sadness over the anticipated separation from their newborns. In heart wrenching descriptions, the participants eloquently gave voice to their sorrow.

Knowing I'll be leaving my baby at the hospital again . . . I'm hurting inside . . . like losing a family member

. . . knowing it's coming to an end. Picturing myself leaving my baby at the hospital. How lonely it will be . . . it's tearing me up inside . . . I'm going to feel empty. Words just can't describe how bad it hurts. I don't want to let him go.

I try not to think about when it's time for me to have my baby. Why? Cause I can't feel those feelings right now. Oh, but deep down my soul they are there . . . I have this pain deep down in my side that just won't come loose.

I got a chance to see my ultrasound yesterday and I could picture being separated from my baby as something very sad and lonely . . . and once more again my heart will be ripped out of me.

Women also grieved their lack of ability to continue their care of and/or contact with their other children. This sorrow was more profound when the children were of an age that precluded their understanding the circumstances and anticipated length of the separation from the mother. As one woman described:

I am so miserable. . . . It may sound bad, but I miss my two-year-old the most. I love all my kids, but my two-year-old did everything with me. And he can't understand why I'm gone and when I'll be back. They say he is talking, and I haven't heard that.

Theme 3. Subjugation

The term "subjugation" exemplifies the physical and emotional distancing experienced by incarcerated women while at the same time suggests the powerlessness experienced by these women within the correctional system. Isolation and lack of personal autonomy were elements of the theme "subjugation" that emerged from the data.

Physical separation from family and friends added to the sense of isolation experienced by pregnant incarcerated women. This physical isolation took the form of distance from their home communities and limited verbal communication with families and friends as a result of a combination of their own or family preferences and prison regulations. One can hear the mothers' aloneness in their words.

I don't have my mom's phone number. I've been unable to reach my sister. I have to go through Melanie. . . . I don't want to have this baby cause I'm afraid I won't see him until I'm released. The baby will be a two hour drive from here. . . . I haven't talked with the kids since I've been locked up. I write, but I don't know if they get my letters.

In addition to concerns regarding the physical separation from family and friends, women expressed extreme discomfort with the emotional isolation experienced within the prison itself. Disclosure of personal information to other inmates made the women who shared this information vulnerable to jibes and sarcasm. Addition-

ally, every interaction involved the possibility of confrontation. The emotional isolation experienced by the women can be heard clearly through this woman's voice.

Prison can be a lonely place. . . . So much runs through my mind and yet still I'm in prison and there is a mask to keep on cause I don't know who is who. It's like any other prisoner. If I'm hurting, I don't trust other prisoners. I hurt alone. You put on a mask. You're not yourself. You have to keep yourself safe—physically and emotionally.

Central to the experience of pregnancy for incarcerated women was the total lack of autonomy that they experienced in relation to the correctional system. They repeatedly expressed the feeling of being controlled by a system that conspired against them. This perceived conspiracy took many forms, including slowness in processing necessary paperwork for a variety of activities, parole denial, delay in receiving mail and their lack of nutritious food choices during pregnancy. Women viewed the forced separation from their newborns as the ultimate example of lack of control over their present situation. One woman verbalized her frustration in this way: "Once they get you here, they don't want to let you go. They will write f---ing tickets, do paperwork slow, lose paperwork."

Lack of control over personal boundaries was a major concern for inmates. Practices they identified as dehumanizing included being frisked by both male and female guards and the possibility of being strip searched after having a "contact" visit or before leaving the facility for any reason, including labor.

No one can understand without being here . . . have to have urine tests . . . undress, squat and they look up in you to make sure you are not hiding urine anywhere . . . ridiculous to treat grown women this way. I know we've all done crimes but it's how they treat you.

Theme 4. Relatedness

Relatedness expresses the positive energy that the women described regarding their experiences of child-bearing in prison. Within this theme are contained the elements of "connectedness to the fetus" and "connectedness to self." Connectedness to the fetus evolved from the participant's statements regarding their involvement with their fetus. Within the element "connectedness to self," women described their ability to persevere in adversity by identifying personal resources.

The element "maternal connectedness to the fetus" incorporated the thematic categories that expressed both the mothers' affective and behavioral responses to the fetus as well as their description of the fetus as companion. As women described their feelings of connectedness to the fetus in the context of being incarcerated, they also expressed apprehension regarding this experience.

Maternal affective responses included the mother's thoughts and feelings toward her fetus. Three-fourths of these women expressed both an awareness of and an affection for their fetuses, regardless of their weeks of gestation at the time of their writing. One woman at 10 weeks gestation wrote: "I'm always thinking of my baby. I love my baby so much." Another at 31 weeks gestation stated: "Do I think about my baby while I am in here? Of course. I even talk to my baby and tell him how much he's loved by his mother." Maternal connectedness to the fetus was sometimes enhanced by incarceration. Several women who had other children reported feeling a special bond with this child because of the "shared" experience of incarceration.

Three participants denied an affective connectedness to their fetus. Social, environmental, and time factors were identified by one soft-spoken woman (18 weeks gestation) as the reasons for her lack of feelings for this baby. In her narrative, her use of the pronoun "it" in reference to her baby accentuated her lack of affective connectedness. She wrote:

Well, being that I have two children already, I can't say I think about my baby much because I don't know him yet. I was most involved with my first pregnancy. I wasn't using (drugs) then. I know it's a part of me, but I'm still getting used to the idea that I am pregnant.

A mother who had another child at home cited her plans for adoption as the reason for her intentional lack of connectedness to her fetus. Coping with an unplanned/undesired pregnancy, this mother (33 weeks gestation) purposely distanced herself emotionally from this child, while still acknowledging interest in the child's well-being. She wrote:

If I was on the streets, I wouldn't have been pregnant this long. I would have had an abortion. . . . I didn't want the baby from the beginning. I always wanted to have one child. I really don't think about my unborn child that much. I'm not attached to it because I'm giving my child up for adoption. . . . It's something that I have to and want to do, so I don't think that much about it. I just don't want to be pregnant any more. I know it sounds crazy, but I have no feelings for this baby. As I stated early it's fine with me. Because that is the way I want it to be. But its great for me cause my child will be getting a new start.

Two of the three mothers who did not express an affective tie to the fetus during time they participated in the study were transferred to an alternative incarceration placement site; thus, it is not known whether their feelings changed over time. The third mother, who planned on relinquishing her infant, maintained an emotional distance from her fetus throughout the pregnancy.

Physical actions taken by the mother to influence the fetus or taken in response to stimulation from the fetus comprise the thematic category of maternal behavioral

responses. Examples of talking to the baby, trying to get the baby to move, and modifying her own behavior in response to perceived fetal preferences can be heard in this mother's voice (31 weeks gestation). "I hold my stomach and rock my baby, I sing, read, and talk to him as if I were actually holding him in my arms."

The third thematic category of the element connectedness to fetus was "fetus as companion." Many mothers reported feeling less alone because of the presence of the fetus. The companionship that the mothers derived from their fetus can be seen in the following comments: "The baby is doing time with me." and "You don't feel alone when you're pregnant and in prison. You know there's someone here to love you." The perceived companionship of the fetus also had negative consequences for the mother, as she anticipated her loneliness after the child's birth. This is reflected in the following passages: "I don't want to let him go. I'm going to miss him being inside of me being able to feel his every move, talking to him, listening to his heartbeat. I'm going to feel empty."

Although pregnancy in prison was viewed by all of the participants as a negative experience, they also identified either personal or situational positive outcomes that occurred as a result of incarceration. These positive consequences together form the theme of "relatedness to self," which consists of the elements self-reflection and goal setting.

Careful consideration of their personal past, present, and future was evident in their journals. Two central aspects of this reflection were self-evaluation and identification of their sources of strengths. Participants were often harshly self-critical. They accepted responsibility for the behavior that resulted in their incarceration, and often saw prison as a buffer between themselves and behaviors that might be harmful to the fetus. As one participant wrote: "I am ashamed of myself. . . . I often beat myself up knowing I put myself and another baby through this again." Another said: "Right now I'm glad I'm clean and I'm glad I'm in prison so that I can stay that way."

Women often referred to the personal strength that was necessary to survive the experience of pregnancy and birth while in prison. Strength was identified as coming from three main sources: the self, the baby, and God. At times, their passages simply reflected personal strength of character, which existed independently of the pregnancy experience. One mom wrote, "In conclusion, I would have to say that it takes a strong person to deal and cope with things that happen in prison." Reliance on a higher power enabled many women to cope with the difficulties associated with being pregnant and incarcerated, for example,

I am tired of being tired (going through the situation of being in prison). I have turned my life over to God. I've

been up, I've been down, and sometimes we have to reach our lowest to get back to life. At this point in my life I just want my respect and learn to live God's ways.

In addition to using the period of incarceration for self-examination, these prisoners also used it as an opportunity to identify desirable personal qualities and to set new goals for their lives. Specific goals and decisions that emerged from the data were to change undesirable behaviors, optimize their opportunity to mother both this and other children, and/or consider life skills necessary for success. This can be seen in the following quotes.

I try to think positively. I try to imagine what it will be like to be with my baby in the world. How to manage money, etc. . . . I'm always thinking of how I will provide for my baby. That's a big factor for me.

Since I'm not married, being a single parent combined with having a record, trying to get jobs will be hard. . . . Will I be able to get a job? I am a waitress. I won't be able to work around alcohol. What will I be able to do?

DISCUSSION

The themes of apprehension, grief, and subjugation that emerged from this study corroborated knowledge gained from previous research focusing on incarcerated women. In other studies, inmates' distrust of the system and grieving over anticipated separation from their newborns consistently emerged as major themes. Isolation, lack of autonomy, high stress levels, and concern about older children are common threads that run through the fabric of all of the studies of incarcerated mothers (15,20,22).

The findings of this study, however, conflict with previous research in one notable way. In contrast to the finding that incarcerated women believed themselves incapable of affecting personal changes in relation to their fetuses (26), the findings of this study indicate that women do modify their thoughts and behaviors when they believe that their fetuses will benefit from these actions. Incarcerated women also describe a high degree of satisfaction from the relationship they develop with their unborn babies.

Somewhat surprisingly, the use of journaling as a method of data collection had positive effects. First, the women were delighted to get the books which were hard bound with lovely printed cloth covers. Second, possession of the books set them apart in a positive way from the other inmates. When other inmates asked if they could get one, the participants were heard to say, "No, this is just for pregnant women in Judy's study." Finally, the fact that they were allowed to keep them between "data collection" points (when this was not initially planned) showed that their wishes were respected. All served to build their feelings of self-worth and commitment to the study.

There were two drawbacks to using journaling as a data collection method, however. The need to journal might have caused a woman who could not write (or who felt that she would be judged by the quality of her writing) to refuse to participate in the study. Similarly, poor vocabulary and writing skills might have impeded the woman's ability to adequately express her experiences. This last drawback was mediated by the interviews that took place with the participants after the journal entries were transcribed. These interviews allowed the researcher to clarify passages and thoughts that she found unclear, while allowing participants to expand on their thoughts. When each participant was given her journal, the researcher emphasized that she was really interested in the woman's thoughts—and that the writing and spelling errors would not be considered. Although never explicitly asked whether they felt intimidated or hindered by having to journal, the researcher never had the impression that the women found journaling to be a problem.

IMPLICATIONS

The women were eager to share their experiences so that those "on the outside" would better understand what it was like to be pregnant and in prison. They wanted both HCPs and the general population to know that although they had committed crimes, they were still mothers and truly grieved their inability to mother previously borne children as well as these newborns. They wanted others to know that it was extremely difficult being in prison while pregnant and hoped that a way might be found for their infants to stay with them. For the most part, they have not been heard. Pregnant incarcerated women are noticeably absent in studies focusing on the childbearing experience.

The loss of incarcerated women's ability to care for their newborns is incredibly painful. As nurses and midwives support mothers who are separated from their newborns due to newborn complications, so too, should they acknowledge and support pregnant and newly birthed incarcerated women as they grieve the loss of contact with their babies.

Because a pregnant inmate's relatedness to self and her fetus during her pregnancy is seldom recognized, this quality has not been tapped. Relatedness may serve as a catalyst, enabling the woman to make positive personal choices that she might not otherwise make such as participation in mothering classes, abstinence from drug and alcohol use, and educational opportunities and employment counseling. These personal choices would maximize mothering skills, increase readiness for re-entry into society, and decrease recidivism rates.

Isolation is antithetical to the support and compan-

ionship necessary for the development of maternal identity. Distant geographic placement of women's prisons physically isolate pregnant inmates from their families and friends while prison policies established to maintain security isolate the women from each other. These factors are not easily changed. Currently, few programs exist that allow the newborn to remain with the mother while she serves the rest of her sentence. When such programs do exist, eligibility criteria exclude many pregnant women from participation. The effectiveness of these programs in enhancing maternal role activities needs to be determined and is in progress in some areas. This is particularly salient if, as some have suggested, recidivism rates of this population might be lowered if maternal role development is enhanced.

Health care providers have an integral role in enhancing the positive experiences for pregnant incarcerated women. In addition to monitoring the physical well-being of the mother and her fetus, HCPs should also assess the psycho-social dimensions of these clients. While it is tempting to suggest that the establishment of prison-based support/educational groups for these women might alleviate their social isolation, their self-imposed seclusion may prove to be a more difficult barrier to overcome than are formidable prison policies.

The rapidly rising number of women imprisoned in the United States underscores the urgency to better understand their experiences. HCPs must talk with pregnant incarcerated women to more fully understand the experience of childbearing in prison. A way must be found to help imprisoned women cope more effectively with the role diffusion/confusion they experience as they live two diametrically opposing roles—those of inmate and mother. Incarcerated mothers must be assisted to identify their strengths and set personal goals. Correction authorities must be encouraged to establish care guidelines that recognize the unique needs of pregnant incarcerated women. Pregnant women make up only 5% of the female prison population; however, their unique situation highlights the need to examine and revise policies and practices that negatively affect all incarcerated mothers as they strive to maintain their parental identity with their child(ren). The task is formidable, but women in prison can no longer remain invisible.

The author would like to thank the women who shared their thoughts and feelings with this researcher "from the outside," and Dr. Jeanne Raisler, CNM, who encouraged and provided feedback on several drafts of this article.

REFERENCES

1. U.S. Department of Justice. Women Offenders. Bureau of Justice Statistics Special Report. (NCJ-175688). Washington (DC): United States Government Printing Office, 1999.
2. U.S. Department of Justice. Prisoners in 1997. Bureau of Justice Statistics. (NCJ-170014). Washington (DC): United States Government Printing Office, 1998.
3. U.S. Department of Justice. Prisoners in 1998. Bureau of Justice Statistics. (NCJ-175687). Washington (DC): United States Government Printing Office, 1999.
4. Rubin R. Maternal identity and the maternal experience. New York: Springer Publishing Company, 1984.
5. Fonagy P, Steele H, Steele M. Maternal representations of attachment during pregnancy predict the organization of infant-mother attachment at one year of age. *Child Dev* 1991;62(5):891-905.
6. Benedek T. The psychobiological aspects of mothering. *Am J Orthopsychiatry* 1956;26(4):272-8.
7. Bibring GL. A study of the psychological processes in pregnancy and of the earliest mother-child relationship. *Psychoanalytical Study of the Child* 1959;14(2):113-21.
8. Deutsch H. Psychology of Women. Vol. 2. New York: Grune and Stratton, 1944.
9. Rubin R. Maternal tasks in pregnancy. *Maternal Child Nurs J* 1975;5(3):143-5.
10. Stainton MC, McNeil D, Harvey S. Maternal tasks of uncertain motherhood. *Maternal Child Nurs J* 1992;20(3-4):113-23.
11. Cranley MS. Development of a tool for the measurement of maternal attachment during pregnancy. *Nurs Res* 1981;30(5):281-4.
12. Muller ME. Development of the prenatal attachment inventory. *Western J Nurs Res* 1993;15(2):199-215.
13. Leifer M. Psychological effects of motherhood: study of first pregnancy. New York: Praeger, 1980.
14. Stainton MC. The fetus: a growing member of the family. *Family Relations* 1985;34(3):321-6.
15. Baunach PJ. Mothers in prison. New Brunswick NJ: Transaction Books, 1985.
16. Barry E. Quality of prenatal care for incarcerated women challenged. *Youth Law News* 1985;6(6):1-4.
17. Bloom B. Mothers in prison: a neglected population. *International Association of Justice Volunteerism in Action* 1990;4(2):1-6.
18. Jose-Kampfer C. Reflections from the inside. *Health PAC Bull* (Winter, 1992).
19. Feinman C. Women in the criminal justice system, 3rd ed. Westport CT: Praeger, 1994.
20. Fogel CI. The mental health of incarcerated women. *Western J Nurs Res* 1992;14(1):30-47.
21. Wilson JS, Leasure R. Cruel and unusual punishment: the health care of women in prison. *Nurs Practitioner* 1991;16(2):32-9.
22. Shelton BJ, Gill DG. Childbearing in prison: a behavioral analysis. *JOGNN* 1989;18(4):301-8.
23. Martin SL, Rieger RH, Kupper LL, Meyer RE, Zaquish BF. The effect of incarceration during pregnancy on birth outcomes. *Public Health Rep* 1997;112:340-6.
24. Egle CC, Miller DE, Granados JK, Ingram-Fogel C. Outcome of pregnancy during imprisonment. *J Reproductive Med* 1992;37(2):131-4.
25. Hufft AG. Psychosocial adaptation to pregnancy in prison. *J Psychosocial Nurs* 1992;30(4):19-23.
26. Fogel CI, Harris BG. Expecting in prison: preparing for birth under conditions of stress. *JOGNN* 1986;15(6):454-8.
27. Shelton BJ, Armstrong F, Cochran SE. Childbearing while incarcerated. *Maternal Child Nurs* 1983;8(1):23-4.
28. Parse RR, Coyne AB, Smith MJ. Nursing Research Qualitative Methods. Bowie, MD: Brady Communications Company, 1985:15.
29. Giorgi A. Convergence and divergence of qualitative and quantitative methods in psychology. In: Giorgi A, Fisher CT, Murray EL, editors. *Duquesne studies in phenomenological psychology: Volume 2*. Pittsburgh, PA: Duquesne University Press, 1975:72-9.
30. Giorgi A. An application of phenomenological methods in psychology.²⁹:82-108.
31. Leininger M. Ethnomethods. The philosophic and epistemic bases to explicate transcultural nursing knowledge. *J Transcultural Nurs* 1990;1(2):40-51.
32. Leininger M. *J Transcultural Nurs*,³¹ 43.
33. Leininger M. *J Transcultural Nurs*,³¹ 114.